PROMPT CARD PACK

11 ChatGPT Prompts for Medical Discussion

Paired with specialised medical-tool demonstrations and visible human checkpoints

AI assists • Professionals decide • Patients remain the purpose

For doctors and pharmaceutical professionals

Use only synthetic/de-identified or approved information

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UNIVERSAL WRAPPER

You are supporting a synthetic healthcare education exercise. You are not the treating clinician and must not replace professional judgement. Do not provide a final diagnosis, prescribe, change a dose, provide reassurance or instruct anyone to delay urgent care. Separate supplied facts, patient-reported information, professional observations, assumptions, unknowns and AI-generated hypotheses. Do not invent findings, medicines, laboratory results, citations or guidelines. Identify red flags and information requiring urgent professional review. Use only the information and approved sources supplied. End with a section titled 'Human decision and verification required'.

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4	Patient-education asset	ChatGPT plus Canva/Gamma; bilingual review may use Bhashini
5	Redesigning an unhelpful clinical alert	ChatGPT discussion prompt
6	Ambient-scribe pilot plan	ChatGPT; compare with Heidi/Nabla governance settings
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PROMPT 1 — Consultation continuity

Turn a synthetic conversation into a reviewable draft without inventing clinical facts.

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TASK
Using the synthetic consultation transcript below, create: (1) a source-grounded draft encounter summary, (2) a problem-information list, (3) medication-reconciliation questions, (4) a plain-language patient summary, (5) three teach-back questions, and (6) follow-up and escalation items. Tag every statement [PATIENT REPORTED], [PROFESSIONAL OBSERVATION], [UNKNOWN] or [NEEDS VERIFICATION]. Do not invent examination findings, diagnosis, medication, dose, investigation, reassurance or follow-up decision. Transcript: [PASTE SYNTHETIC TRANSCRIPT]

Expected output	Human checkpoint	Failure to discuss
Tagged draft note, patient summary, questions and escalation items	Reconcile facts and medicines; check red flags, consent, local language and record policy before sign-off.	Invented findings or a polished draft copied into the record without review.

DEBRIEF QUESTION

What attention did the tool return—and what new verification work did it create?

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PROMPT 2

Diagnostic reasoning without outsourcing diagnosis

Expose hypotheses, missing information and urgency.

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TASK
Using only the synthetic case below: (1) write a one-sentence problem representation without declaring a diagnosis; (2) group hypotheses as common/likely, dangerous must-not-miss, and alternative explanations; (3) for each, list supporting information, contradictory information, missing information, red flags, time sensitivity and evidence that must be verified; (4) identify the five questions or observations most likely to change urgency; and (5) state what a licensed clinician must decide. Do not recommend treatment or invent probabilities. Case: [PASTE SYNTHETIC CASE]

Expected output	Human checkpoint	Failure to discuss
Structured hypotheses and uncertainty map	Check urgent red flags, primary evidence, local protocol and patient context; clinician owns diagnosis and disposition.	A plausible list mistaken for a diagnosis or a reason to delay escalation.

DEBRIEF QUESTION

Did the output widen attention or create noise?

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PROMPT 3	Medication-reconciliation discussion

Surface possible medicine issues that require verification.

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TASK
Review the synthetic medication information below. Do not recommend starting, stopping or changing any medicine. Create a verification checklist covering possible duplication, interaction, allergy conflict, renal/hepatic considerations, pregnancy/lactation if relevant, indication uncertainty, adherence, OTC/supplement/traditional-medicine use, monitoring and missing name/strength/formulation/route/frequency. Separate confirmed facts, potential issues to verify and missing information. List questions for the patient and prescriber/pharmacist, plus the approved drug-information sources that must be checked. Information: [PASTE SYNTHETIC LIST]

Expected output	Human checkpoint	Failure to discuss
Verification checklist and escalation questions	Licensed prescriber/pharmacist checks labels, formulary, renal/hepatic context and local protocol.	Model invents an interaction or silently changes a dose.

DEBRIEF QUESTION

Which issue is a signal—and which is a verified fact?

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PROMPT 4 | Patient-education asset

Convert approved evidence into actionable plain language.

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TASK
Using only the approved source excerpt below, draft a one-page patient handout for [AUDIENCE] at class 6–8 reading level. Include a short title, five practical actions, medicine-taking reminders without products or doses, monitoring, routine follow-up, three urgent red flags, three teach-back questions, source, review date, reviewer placeholder and AI-assistance disclosure. Mark unsupported statements [REVIEW]. Provide a draft Hindi version, marking medical terms for bilingual professional review. Do not invent statistics, guarantees or clinical advice outside the source.
Source: [PASTE APPROVED SOURCE]

Expected output	Human checkpoint	Failure to discuss
Plain-language handout with red flags and teach-back	Clinical meaning, translation, accessibility, source, version and reviewer approval.	Unsafe simplification, mistranslation or an unsupported promise.

DEBRIEF QUESTION

Can the patient act safely—and is routine advice separated from urgent action?

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PROMPT 5 — Redesigning an unhelpful clinical alert

Apply the SepsisLab design lesson to a synthetic alert.

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TASK
Evaluate the synthetic alert below without making a patient-specific treatment recommendation. Analyse timeliness, false-positive burden, false-negative risk, explanation, uncertainty, actionability, workflow interruption, override, escalation and audit trail. Redesign it to show what changed, why it appeared, current uncertainty, information that could reduce uncertainty, required professional review, override/documentation path, pilot metrics and stop criteria.
Alert: [PASTE SYNTHETIC ALERT]

Expected output	Human checkpoint	Failure to discuss
Before/after alert specification and pilot measures	Clinical governance approves intended use, escalation, thresholds and monitoring.	A more attractive alert that still arrives late or triggers unowned action.

DEBRIEF QUESTION

Does the redesign help before the decision—or simply explain after it?

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PROMPT 6 | Ambient-scribe pilot plan

Design a controlled implementation with quality assurance.

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TASK
Design a six-week controlled ambient-scribe pilot. Include intended and excluded uses; patient notice/consent; minimum necessary data; clinician and scribe-reviewer responsibilities; note sampling; error taxonomy; hallucination and omission monitoring; patient-experience, clinician-time and after-hours documentation measures; safety escalation; rollback; stop criteria; and final continue/redesign/stop decision. Do not assume time saving proves safety.

Expected output	Human checkpoint	Failure to discuss
Six-week pilot charter and measurement plan	Privacy, clinical governance, IT/security and accountable service owner approve the pilot.	Plug-and-play rollout without consent, sampling or stop rules.

DEBRIEF QUESTION

Who reviews the reviewer—and what error stops the pilot?

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PROMPT 7

Pharma medical-information and safety triage

Separate scientific response, off-label handling and immediate safety escalation.

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TASK
Analyse the synthetic medical-information interaction. Create: the exact unsolicited scientific question; minimum information needed; approved sources; a neutral response outline; statements requiring medical/legal/regulatory review; possible off-label elements requiring medical-information handling; possible adverse-event or product-quality information requiring immediate escalation; follow-up commitments; and documentation requirements. Do not create promotional claims, assess causality, minimise an event or delay escalation. Interaction: [PASTE SYNTHETIC INTERACTION]

Expected output	Human checkpoint	Failure to discuss
Neutral inquiry plan plus explicit safety/PQC escalation flags	Medical information, pharmacovigilance and MLR follow the approved SOP; safety escalation is immediate.	A polished response that misses or delays an adverse-event report.

DEBRIEF QUESTION

Which part belongs to medical information, PV, product quality or MLR?

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PROMPT 8 | Explainable-AI vendor evaluation

Turn marketing claims into an auditable due-diligence conversation.

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TASK
Evaluate the medical-AI product description. Build a due-diligence table for intended use/user, target population, setting, validation population and comparator, sensitivity/specificity, false positives/negatives, subgroup performance, external validation, explainability method and clinical meaning, workflow integration, override, audit trail, data location/retention, jurisdiction-specific regulatory status, evidence gaps, India applicability, vendor questions, reasons to pilot and reasons not to pilot. Do not infer approval or effectiveness from marketing language.
Description: [PASTE]

Expected output	Human checkpoint	Failure to discuss
Vendor-question list and pilot/no-pilot rationale	Clinical, regulatory, privacy, security and procurement reviewers verify original evidence and status.	Decorative heatmaps treated as proof of correctness.

DEBRIEF QUESTION

Is the explanation clinically meaningful or merely persuasive?

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PROMPT 9 — Socratic medical teaching

Use AI to strengthen reasoning rather than reveal an answer immediately.

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TASK
Act as a Socratic facilitator for the synthetic case. Do not reveal the answer immediately. Ask the learner to create a problem representation; propose common, dangerous and alternative hypotheses; identify evidence for/against; identify missing information and urgency; challenge one assumption; introduce one verified source; ask the learner to revise; then provide feedback with a transparent rubric and identify unsupported AI-generated information. Case: [PASTE]

Expected output	Human checkpoint	Failure to discuss
Staged questions, revised reasoning and rubric feedback	Faculty verifies content and preserves learner effort, dialogue and originality.	Instant answers reduce productive struggle and create look-alike submissions.

DEBRIEF QUESTION

Did the AI promote reasoning or replace it?

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PROMPT 10 — Professional projection

Draft a credible evidence-based professional post with visible boundaries.

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TASK
Using only the approved evidence below, draft a 150–200 word professional post for [DOCTOR / MEDICAL-AFFAIRS PROFESSIONAL] to [EDUCATE PATIENTS / EDUCATE PEERS / DISCUSS AN EVIDENCE GAP]. Include one evidence-based message, one practical implication, one limitation, a visible source, AI-assistance disclosure and an invitation to professional discussion—not individual treatment advice. Do not add product promotion, off-label claims, exaggerated certainty or authority beyond the stated role. Evidence: [PASTE]

Expected output	Human checkpoint	Failure to discuss
Concise, sourced and bounded professional post	Clinical/MLR review, citation, disclosure, version owner and publication approval.	Authority signalling, promotion or advice without patient context.

DEBRIEF QUESTION

Does the post prove usefulness before claiming authority?

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PROMPT 11 Rapid evidence map

Create a reproducible starting map without fabricating citations.

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TASK
Convert the question into a transparent rapid-research workflow. Produce PICO; concepts and synonyms; a draft PubMed Boolean query; inclusion and exclusion criteria; evidence-extraction fields; citation-verification checklist; bias/applicability questions; uncertainty log; and a statement of what this rapid review cannot conclude. Do not invent citations. Label every citation [UNVERIFIED] until located in PubMed, a registry or the original publication.
Question: [PASTE]

Expected output	Human checkpoint	Failure to discuss
Search strategy, extraction frame and uncertainty log	Every citation and extracted field is verified in the original source; methods and limitations are documented.	A fluent synthesis with fabricated or mischaracterised studies.

DEBRIEF QUESTION

Which evidence was actually verified—and why is this not a systematic review?

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